

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: JB Period: September 23 to September 30, 2024 Report Date: October 1, 2024 Coverage: Bed: 135/182h (74.2%)
Chair: 145/182h (79.7%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Sep 23 to Sep 30 ■ HR ■ Breathing

V.High HR (#1)					#4	V.High HR (#1)				
Sep 23		Sep 24		Sep 25	Sep 26		Sep 27	Sep 28	Sep 29	Sep 30
Episodic Events		Total Hours		Highest Burden			Episodes/day		Coverage	
33		103h		Very High Heart Rate			4		100%	

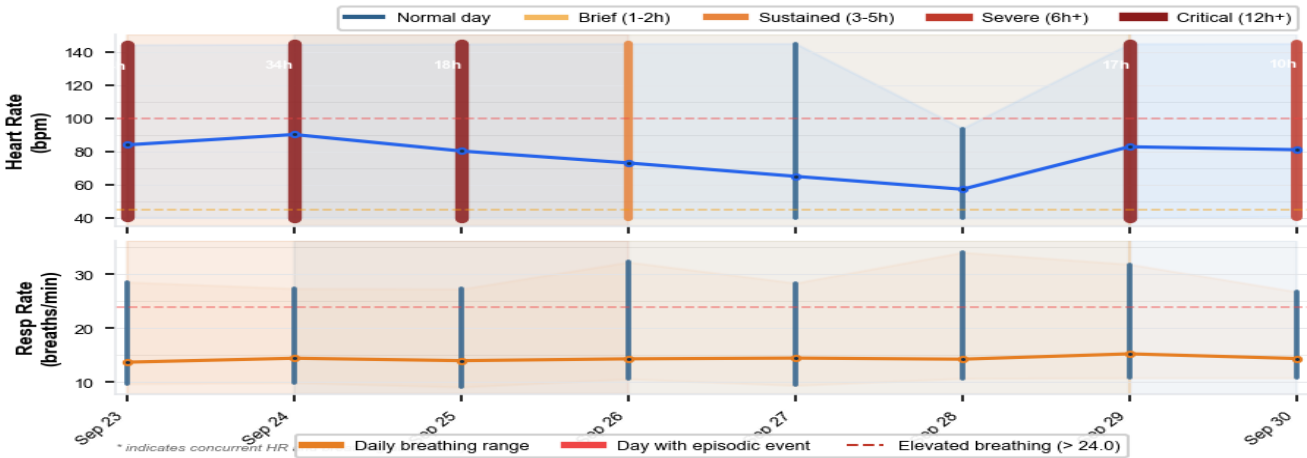
Major Findings: Sustained very high HR (avg 76, peak 145)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Heart Rate	145 bpm	61h	24h	1	76 bpm	Sep 23 to Sep 29
2	High Heart Rate	145 bpm	8h	2h	2	82 bpm	Sep 23 to Sep 26
3	High Heart Rate	130 bpm	1h	1h	1	83 bpm	Sep 29
4	Low Heart Rate	40 bpm	31h	5h	2	76 bpm	Sep 24 to Sep 30
5	Elevated Heart Rate	113 bpm	1h	1h	1	83 bpm	Sep 29
6	Elevated Breathing	27 brpm	1h	1h	1	15 brpm	Sep 29

The P5 to P95 heart rate spread was 100 bpm (43 to 143), indicating significant cardiac variability.

Clinical Pattern Observations:

- 1. **Sustained finding:** 24h continuous Very High Heart Rate beginning Sep 23.
- 2. **High variability:** HR spread 100 bpm (43 to 143).
- 3. **Continuous pattern:** Episodic events on 6 of 8 days (75%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.